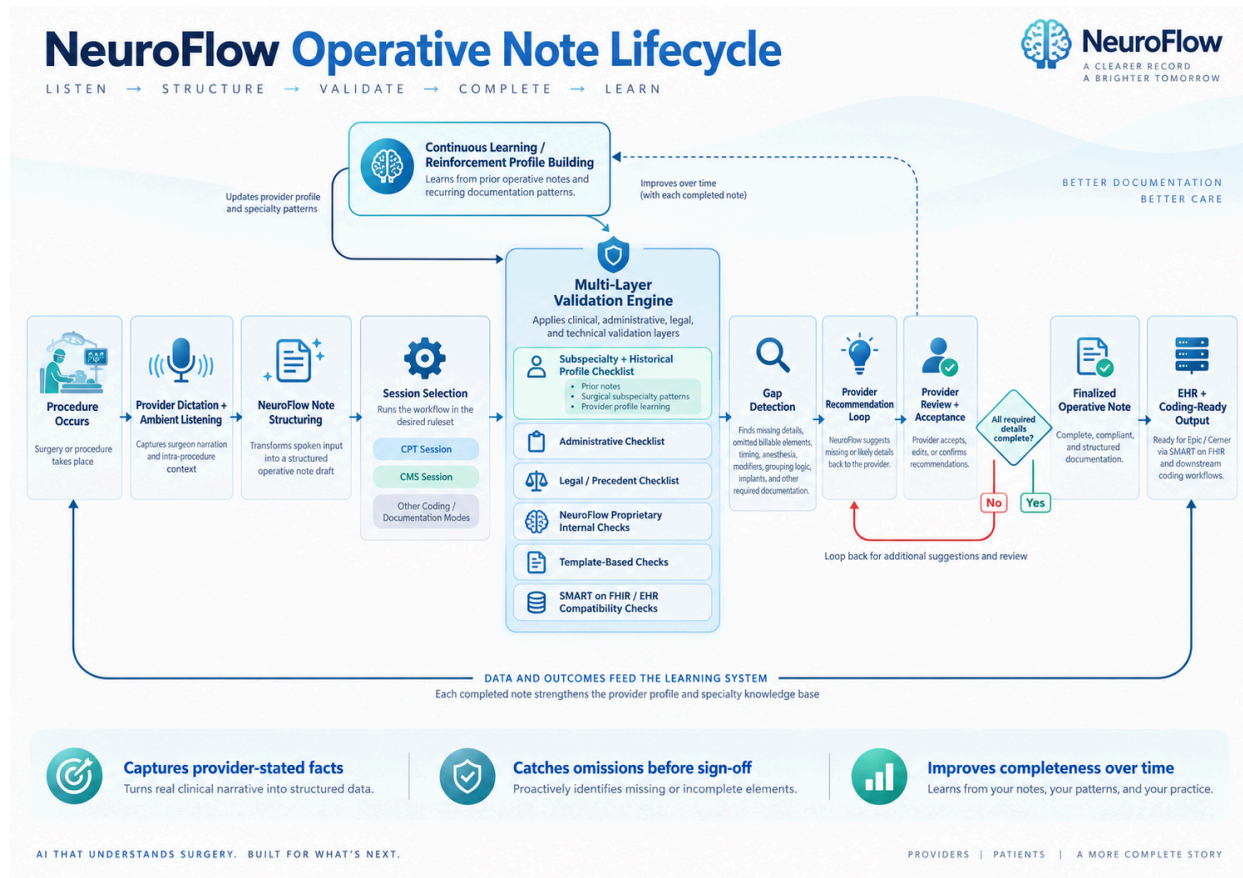


App1 - AI Operative Note Suite (The Capture)

“We are not selling an AI scribe. We are selling surgical capacity and revenue capture at the point of care.”



Value of Operative Note Suite

- **Reduce operative note completion from 30–60 minutes to approximately 5–10 minutes per case.** The surgeon dictates the facts of the procedure while NeuroFlo assists with structuring, reviewing, and completing the operative note.
- **Increase Surgical Capacity:** Reducing documentation time returns clinical hours to surgeons. The model in the document shows that reclaiming just 30 minutes per day across 20 surgeons creates approximately **2,500 additional surgical hours annually**, approximately **\$10 million in additional top-line revenue capacity**, and approximately **\$2 million in net profit at a 20% operating margin**.

- **Prevent Charge Leakage at the Point of Care:** Uncaptured procedures, omitted implants, untracked OR time, and missing secondary codes permanently erase up to 3% of a hospital's surgical revenue. NeuroFlo enforces real-time completeness while the surgeon's knowledge of the case is fresh, capturing 100% of billable components before sign-off to recover millions in unbilled care. Preventing revenue from being lost simply because it was never documented.
- **Accelerate the Path to Billing :** By instantly translating the AI-perfected operative note into CPT, ICD-10, and HCPCS codes, NeuroFlo shrinks the DNFB(Discharged Not Final Billed) and DFNC (Discharged Not Final Coded) charge lag from 7 days to under 24 hours. For a hospital processing \$5M daily, this unlocks **\$30,000,000 in immediate liquidity**. Source: [DNF in Healthcare](#)
- **Reduce Documentation Burden & Physician Burnout:** Less time spent writing, reviewing, correcting, and revisiting operative notes removes a repetitive administrative burden from already demanding surgical workloads.

Problems Solved

- **Point-of-Care Completeness:** NeuroFlo analyzes the surgeon's documentation against historical data, legal requirements, hospital requirements, administrative requirements, physician-specific preferences, note structure, and complex surgical subspecialty requirements, prompting the surgeon when information is missing, inconsistent, or insufficient before finalization.
- **Reduced Documentation Burden & Physician Burnout:** Less time spent writing, reviewing, correcting, and revisiting operative notes removes a repetitive administrative burden from already demanding surgical workloads, allowing surgeons to spend more of their time on clinical care.
- **Real-Time Completeness Analysis:** NeuroFlo analyzes the operative note while it is being created and identifies missing critical parameters such as anesthesia time blocks, implants, devices, secondary anatomical repairs, procedures, and other billable details before the surgeon signs.
- **Intelligent Subspecialty Alignment:** NeuroFlo identifies complex procedures and surgical subtypes and cross-references the documentation against CPT (Current Procedural Terminology), ICD-10-CM (International Classification of Diseases, 10th Revision, Clinical Modification), CMS (Centers for Medicare & Medicaid Services), and HCPCS (Healthcare Common Procedure Coding System) requirements.
- **Historical Surgical AI and Reinforcement Learning Intelligence:** NeuroFlo uses AI & RL to learn and gather signals from similar surgeries and previous operative notes over time, using historical procedure patterns to identify information that would normally be expected but may be missing from the current note.

- **Revenue Guardrails Before Sign-Off:** If a required clinical or billing variable is missing, NeuroFlo identifies it while the surgeon can still correct the documentation, rather than leaving a downstream coder to work from an incomplete report.
- **Move Beyond Passive Transcription:** Unlike other vendors, NeuroFlo does not simply record what the surgeon says. It actively evaluates the operative note for clinical, structural, financial, and procedural completeness before it becomes the permanent source document used downstream.

